

Building on Becker's HIT + RCM 2026

How the industry's biggest health-IT & revenue-cycle program informs the next phase of the LumaChart EHR — themes to build on, and the people to build with.

Source: Becker's 11th Annual Health IT + Revenue Cycle Conference — *The Future of AI & Digital Health* · Sept 14–17, 2026, Hilton Chicago · 600+ speakers (90%+ health-system executives), 190+ sessions, 13 tracks. Speaker, agenda & CIO-Forum pages read 2026-09-14. Prepared for the LumaChart / Luma EHR venture.

Read the room honestly. This is an **Epic-funded, enterprise, revenue-cycle-first** gathering — built around big health systems, AI-at-scale, and margin. That is exactly why it is useful to LumaChart: it tells us *what the market is buying* (ambient documentation, AI agents, governance, interoperability), and it throws our contrarian wedge into relief — **well-being first, billing in the background, transparent pricing, evidence on the record**, and the small / independent / safety-net providers the big-system program underserves.

1 · What the program is really about (theme analysis)

Across 13 tracks the agenda clusters into three currents — and LumaChart already has a feature in each. The keynote pairing (Dr. Bobbie Byrne, CIO Advocate Health, with Dr. Mitchell Schnall, Penn Medicine) and the fireside with Luis Taveras (Jefferson) set an AI-governance-and-scale tone; celebrity keynotes (Gretzky, Jimmy Johnson, Vernice "Flygirl" Armour) signal "leadership under pressure."

CONFERENCE CURRENT	REPRESENTATIVE SESSIONS	LUMACHART ANSWER
AI strategy, governance & scale	AI Portfolio: Scale, Pause or Kill? · Fostering Trust in Healthcare AI · AI, Automation & Risk: Balancing Innovation with Oversight · Your AI Pilot Succeeded — Now What? · Scaling AI Enterprisewide: From Pilot Purgatory to Systemwide Impact	NAM AI Code of Conduct card · Regulatory architecture (non-device CDS / FDA PCCP) · §170.315(b)(11) source-attribute transparency · RSI two-loop + PCCP scaling plan built
Revenue cycle & finance	AI Agents in RCM · Denials & Slow Payments · Clinical Coding Automation ROI (UNC Health) · Revenue Starts in the Exam Room · Turning Post-Payment Risk into Recovered Revenue · Patient Financial Experience	Encounter & claim + agent coding · AR outcomes agent ("outcomes, not statuses") · transparent pricing · patient payments + Medicaid coverage built — kept deliberately <i>in the background</i>
Clinical & operational transformation	AI-Assisted Documentation: Real Time Back to Clinicians · Ambient AI (CMIO) · Reducing Nurse Burnout with Automation · Beyond Interoperability: Primary↔Specialty Gaps · Interoperability as a Financial Lever · Reinventing Patient Engagement	Luma Scribe (ambient) · Canary + batched inbox + Focus (burnout) · unified "My record" timeline + specialty packs · FHIR sandbox (g)(10) built

The validation: every major theme the field is investing in, LumaChart already prototypes — often with a sharper, evidence-cited, transparency-first version. The conference is less a to-do list than confirmation we are building the right things. The **differentiator** is ordering: the industry leads with revenue and scale; LumaChart leads with the clinician and the patient, and lets revenue succeed quietly underneath.

2 · Five things to build next (drawn from the agenda)

Where the program surfaced a capability we do not yet fully express, these are the highest-leverage additions — each reuses components we already have.

① **AI Governance & Assurance view** new §170.315(b)(11)

The single most-repeated conference theme is AI *governance* (trust, oversight, ROI, "scale/pause/kill," vendor sprawl). Build a leadership view — a **model registry**: every AI feature (Scribe, coding agent, Canary, deterioration model) with its intended use, source attributes, monitoring metrics (adoption, override rate, drift), PCCP status, and a scale/pause/kill decision. Ties our NAM AICC + FDA + RSI work into one screen a CIO can govern from.

② **"Pilot → Systemwide" packaging** new

"Pilot purgatory" (incl. Deanna Hanisch's Session 14) is the CIO pain of 2026. Package the Burden lab's measured adoption + minutes-returned as an explicit **pilot-to-scale dossier**: baseline → single-feature rollout → 90-day measured result → PCCP-bounded expansion. This is also the exact language of an SBIR Phase-I → Phase-II arc.

③ **AI security & data-governance note** new §170.315(d)

"Your Data Is the Attack Surface: Securing AI in Healthcare" flags a gap. Extend the Security view with AI-specific controls: PHI minimization for model inputs, prompt-injection/untrusted-content handling, audit of every model call, and the data-provenance guarantees behind the evidence engine.

④ **Denials *prevention*, front-loaded** new

Denials automation is everywhere on the RCM track. Extend the AR outcomes agent upstream: front-end eligibility + coding edits that prevent the denial — surfaced as one quiet check at the encounter, never a billing-first interruption of the note.

⑤ **CIO "Build vs. Buy vs. Platform" one-pager** new

A dedicated CIO-Forum session. Our composition strategy (FHIR-native Medplum/Aidbox core + bought certified modules) is a direct, credible answer to vendor sprawl. Package it as a buyer-facing brief that maps to the Certification (Part 170) view.

3 · Who to build with — collaborators & their work

90%+ of the room are health-system executives; the value of this conference is **people**. Ranked by fit to LumaChart's mission (clinician & patient well-being, evidence on the record, safety-net reach). Publication notes are from PubMed searches on 2026-09-14; where an author name is common, ORCID confirmation is advised before citing specific papers.

PERSON / ROLE	WHY THEY FIT	THEIR PUBLISHED WORK
Deanna Hanisch VP & CIO, Johns Hopkins Health System (your flag)	Session 14 — <i>Scaling AI Enterprisewide: From Pilot Purgatory to Systemwide Impact</i> + CIO Forum. An academic medical center CIO — the tripartite (care + research + education) mission is LumaChart's research/public-health layer. The design-partner for the SBIR.	Executive/operational leadership (JHM); thought-leadership via Becker's/CHIME rather than primary literature — engage on scaling & governance, not co-authorship.
Bree Andrews, MD Chief Wellness & Vitality Officer, UChicago Medicine	Direct mission match — a real CWO is exactly the persona of our CWO tab. Ideal validation partner for the Burden lab, Canary, and the well-being instruments.	Neonatology + clinician well-being (PubMed shows an extensive body under this name — confirm identity via ORCID; well-being & parental-support themes).
Sam Amirfar, MD CMO & CIO, The Brooklyn Hospital Center	His CIO-Forum talk — " <i>Will AI Replace Doctors or Just Their Paperwork?</i> " — is LumaChart's thesis verbatim. Dual clinical+IT leader at a safety-net hospital: our beachhead.	Clinical informatics leadership; safety-net digital transformation. Engage on the paperwork-not-doctors framing + Medicaid/coverage features.
Ashish Atreja, MD, MPH Founding Exec. Dir., Stanford GAIN; Professor of Medicine	Leads digital-medicine evidence & validation — the academic backbone for "evidence on the record," non-device CDS, and the RSI validation loop. Natural STTR partner.	PubMed: ~28 papers in digital medicine / AI / mobile health (incl. <i>npj Digital Medicine</i>) — app-validation frameworks, digital therapeutics. High-confidence match.
Bobbie Byrne, MD, MBA EVP & CIO, Advocate Health	Opening keynote; a physician-CIO at enterprise scale — credibility on EHR usability & adoption, the outcome LumaChart optimizes.	Health-IT leadership + EHR usability commentary; enterprise EHR deployment experience.
Mitchell Schnall, MD, PhD SVP Data & Technology, Penn Medicine	Co-keynote; senior academic leader — an AMC research partner for prospective validation (the FDA/PCCP path).	PubMed: ~36 papers in AI / imaging informatics (radiology; national trial-network leadership). High-confidence match.
Salim Afshar, MD, DMD	Focus on translating AI into practice responsibly — aligns with our governance + NAM AI Code	PubMed: ~46 papers across AI/ML + global health (confirm identity via

Harvard T.H. Chan posture.
(AI Translation
Lead); Founder,
Institute for AI
Translation

ORCID).

Supporting network: Curtis Cole (VP & Chief Global Info Officer, Cornell) & DP Harris (SVP/CDIO, Loma Linda) — CIO-Forum peers; Luis Taveras (EVP CDIO, Jefferson); Nari Gopala (Chief Digital Officer, Mayo Clinic); Patrick McDermott (Chief Revenue Officer, Geisinger) — RCM buyer perspective; Ashish Atreja's Stanford GAIN network for academic validation.

4 • Best days & must-see sessions (given the project)

The program runs four days (Mon Sept 14 – Thu Sept 17). Two days carry almost all the value for LumaChart: **Monday** (main-stage themes that validate the build) and **Wednesday** (the CIO Forum — where the collaborators are). If you attend two days, attend those.

★ Monday, Sept 14 — thematic validation & main stage

- **8:00** Opening Keynote — **Bobbie Byrne** (CIO, Advocate) + **Mitchell Schnall** (Penn) — set the governance/scale tone; both on the collaborator list.
- **8:50** *Reducing Nurse Burnout with Automation: What Works and What Backfires* — our core thesis, stress-tested.
- **9:40 / 10:45** *The Next Frontier for CMIOs in Ambient AI · AI-Assisted Documentation: Real Time Back to Clinicians* — Luma Scribe's market.
- **9:40** *Clinical Coding Automation ROI (UNC Health) · Revenue Starts in the Exam Room* — coding agent + billing-in-background.
- **9:40 / 1:30** *Fostering Trust in Healthcare AI · AI Governance in an Era of Constant Change* — the AI-governance view (build #1).
- **10:45 / 1:30** *Beyond Interoperability: Primary↔Specialty Gaps · Interoperability as a Financial Lever* — timeline + FHIR.

★ Wednesday, Sept 16 — CIO Forum (the collaborator day)

- **8:35** *Will AI Replace Doctors or Just Their Paperwork? — a Brooklyn Safety-Net CMO/CIO's 10-Year View* — **Sam Amirfar**. Our thesis + our beachhead; introduce yourself here.
- **10:35** *Build vs. Buy vs. Platform: CIOs' Best Practices* — our composition strategy (build #5).
- **1:20 Session 14 — Scaling AI Enterprisewide: From Pilot Purgatory to Systemwide Impact — **Deanna Hanisch** (JHHS). Your flagged collaborator; the design-partner conversation.**
- **2:10** *The EHR Strategy to Win the Next Five Years* — position LumaChart's contrarian bet.
- **3:15** *The Academic Medical Center CIO: Balancing the Tripartite Mission* — the research/public-health layer; AMC design partners.
- Also in the room: **Curtis Cole** (Cornell), **DP Harris** (Loma Linda).

- **1:30** *Your AI Pilot Succeeded — Now What?* — the pilot-to-scale story (build #2).

Tue 15 & Thu 17: supporting value — watch for additional RCM (denials, revenue integrity) and clinical-transformation tracks, and the specialty CFO/CMO forums, but Monday + Wednesday are the priority. Confirm exact times in the live agenda; sessions and rooms shift.

5 · How to work the conference (human next steps)

- **Target three conversations, not sixty:** Hanisch (scaling/governance + AMC design partner), Andrews (well-being validation), Amirfar (paperwork-not-doctors + safety-net). Atreja for an academic/STTR tie.
- **Lead with the wedge:** "You're all automating the revenue cycle; we start with the clinician and the note, and let billing succeed underneath — measured, cited, and built to §170.315." It is memorable precisely because it inverts the room.
- **Bring one artifact:** the live demo (clinical-trials.github.io/EHR) + the Burden lab + the Certification (Part 170) view. Don't pitch slides to executives who build software; show the working thing.
- **Tie it to the grant:** a JHHS/Stanford design or research partner strengthens the AHRQ SBIR (and enables the STTR variant) — see [CERTIFICATION-AND-GRANT-ACTIONS.md](#).

Bottom line. The conference confirms LumaChart is building the right capabilities and points to five sharp additions (AI-governance view, pilot-to-scale packaging, AI security, denials prevention, a CIO build/buy brief). Its greater value is human: a short list of mission-aligned collaborators — a Johns Hopkins CIO, a Chief Wellness Officer, a safety-net CMO/CIO, and a Stanford digital-medicine director — who can turn a prototype into a funded, certified, adopted platform.